

Libya’s silent epidemic: hypertension control is the best first investment against non-communicable diseases

Non-communicable diseases (NCDs) cause **76%** of Libya’s disease burden. Premature NCD deaths have not fallen since 2000, and Libya is not on track for its 2030 SDG target. Controlling high blood pressure is affordable, proven and fast to scale.

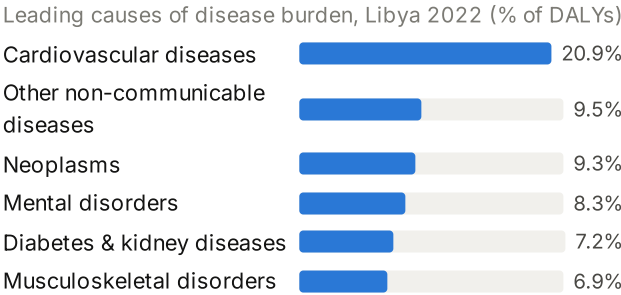


Key messages

- Nearly **1 in 5** Libyans aged 30 will die from heart disease, cancer, diabetes or chronic lung disease before 70 (19.8% in 2021, 20.0% in 2000).
- 42.7%** of adults aged 30–79 have hypertension (about 1.5 million people). **52%** of them do not know it, and only **11%** have it under control.
- Money is not the main barrier: Libya spends more per person on health than its North African neighbours (US\$470), but spending swings sharply from year to year.
- Raising control to 50% would cost about **US\$1,949 per healthy life-year gained** — well below Libya’s GDP per capita (US\$6,569).

1 • The problem

Cardiovascular disease is Libya’s leading cause of ill health (20.9% of all DALYs), and 91% of that burden comes from premature death. Among its North African neighbours, Libya has the highest levels of diabetes (28%), hypertension (42.7%) and physical inactivity (46%), and one third of adults are obese (34%). To meet SDG 3.4 (a one-third cut in premature NCD mortality by 2030, to 13.6%), Libya would need to reduce this rate by 4.1% a year. The current pace is 0.5%.



2 • The care gap

Among Libyans with hypertension (2019):



Most of the loss happens before treatment starts: half of people with hypertension are never diagnosed. This gap can be closed in primary care with simple tools: a blood-pressure cuff, a standard protocol and a reliable medicine supply.

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github.com/nathee-rne-hub/libya-ncd-burden

3 · Why act now

Libya's NCD burden keeps rising, and every few years a shock lands on top of it. Violence reached 21% of all DALYs in 2011, COVID-19 pushed respiratory infections to 21% in 2021, and injury DALYs rose about 9× in 2023, the year of the Derna flood. People with uncontrolled hypertension and diabetes are the most vulnerable when services are disrupted. Health spending per person is the highest among its neighbours (80% publicly funded, 19% out of pocket), but it swings sharply from year to year (standard deviation of annual change ≈ 28%), so long-term programmes need protected funding.

4 · The investment case

We modelled raising hypertension control from 11% to 50% of adults with hypertension over five years:

≈586,000 more people with blood pressure under control	US\$145 M over 5 years (≈ US\$29 M a year)	≈5,913 heart attacks and strokes averted	≈64,353 healthy life-years (DALYs) gained
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Net cost is **US\$1,949 per DALY averted** (95% uncertainty range US\$646–5,321). The probability that the programme is cost-effective at one times GDP per capita is **99%**. These estimates are conservative: they count only five years, and only the acute-care savings from averted events.

5 · Recommendations

- 1. Adopt one simple national treatment protocol** in primary care, based on the WHO HEARTS package: fixed-dose medicine combinations and team-based care with nurses and pharmacists.
- 2. Find the missing half.** Measure blood pressure at every primary-care visit and in pharmacies, and run yearly screening campaigns.
- 3. Protect the medicine supply** with a ring-fenced budget line and pooled procurement for essential blood-pressure and diabetes medicines, so supply survives budget swings.
- 4. Measure what matters.** Run a new national STEPS risk-factor survey, set up a simple hypertension registry, and track the control rate as a national indicator.
- 5. Plan for continuity in crises.** Build NCD medicine stocks and patient lists into emergency plans for conflict, epidemics and floods.

About this analysis

Data: WHO Global Health Observatory (2000–2024), GBD 2023 (IHME, 2024) and UN World Population Prospects 2024, compared with Tunisia, Algeria, Egypt and Morocco. The economic model uses published costs (WHO HEARTS, Moroccan hospital studies) and effects (Ettehad et al., Lancet 2016), with 5,000-draw probabilistic sensitivity analysis. Estimates are modelled and carry wide uncertainty; one input (programme overhead) is still an assumption. Code, data and methods: github.com/nathee-ne-hub/libya-ncd-burden

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